



Smarter Patient Scheduling at Benevis

Hybrid AI + Human Support

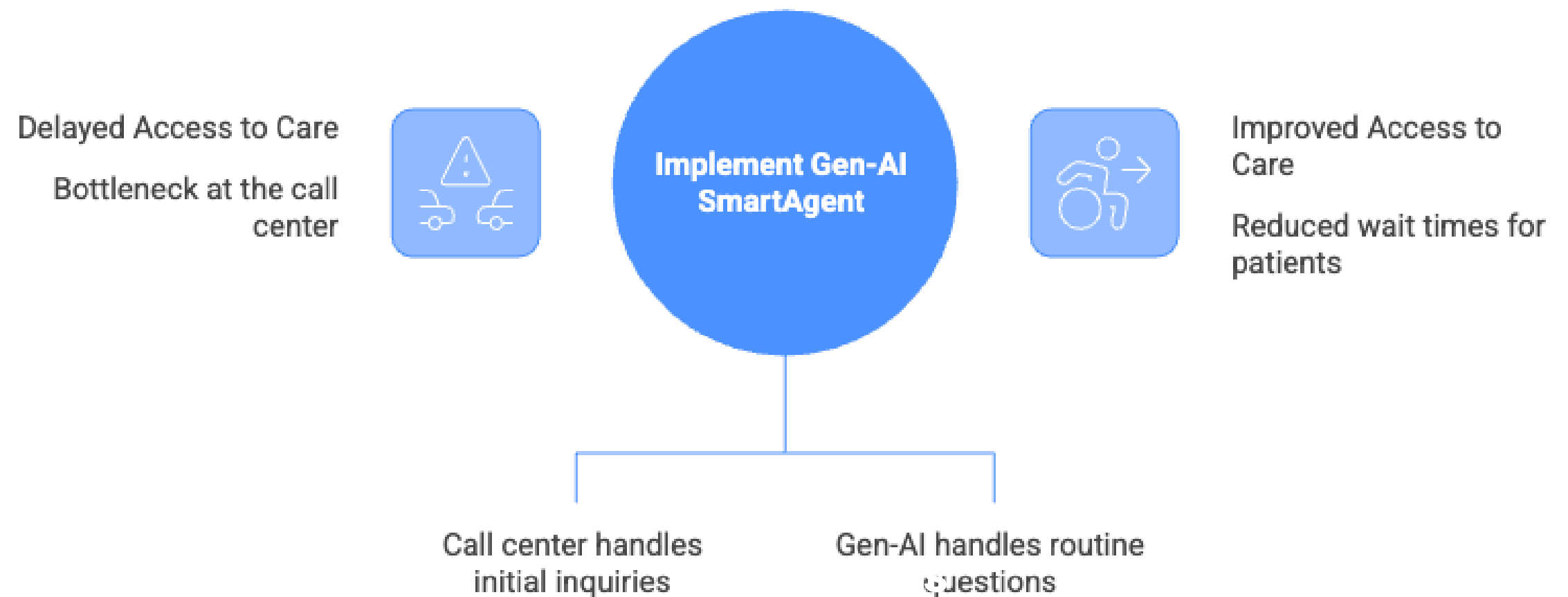
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Background

- Benevis uses its call centre as the main 'front door' for patient access (scheduling + inquiries).
- Growth + uneven demand → missed calls / long waits / inconsistent responses across locations.
- Benevis explores IntelPeer SmartAgent (Gen-AI) to automate routine administrative interactions.
- This is a digital transformation decision, not just a tech install (trust + compliance + stability).

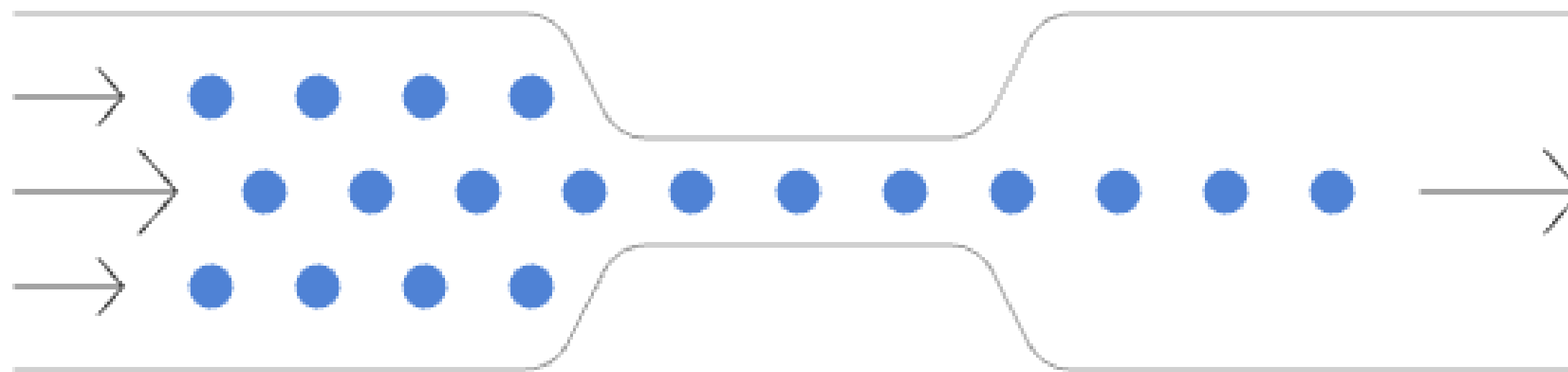
Improving Patient Access to Care



Problem Statement

Benevis cannot scale its call-center-based patient scheduling model to reliably handle rising and highly variable call demand during peak periods.

High call volume overwhelms system, causing delays



Issues (Identification + Prioritization)

Scalability breakdown during peak demand (core performance gap)



Technology & data readiness gap (fragmented workflows/data limits scale)



Operational efficiency & cost pressure (routine work stays human-handled)



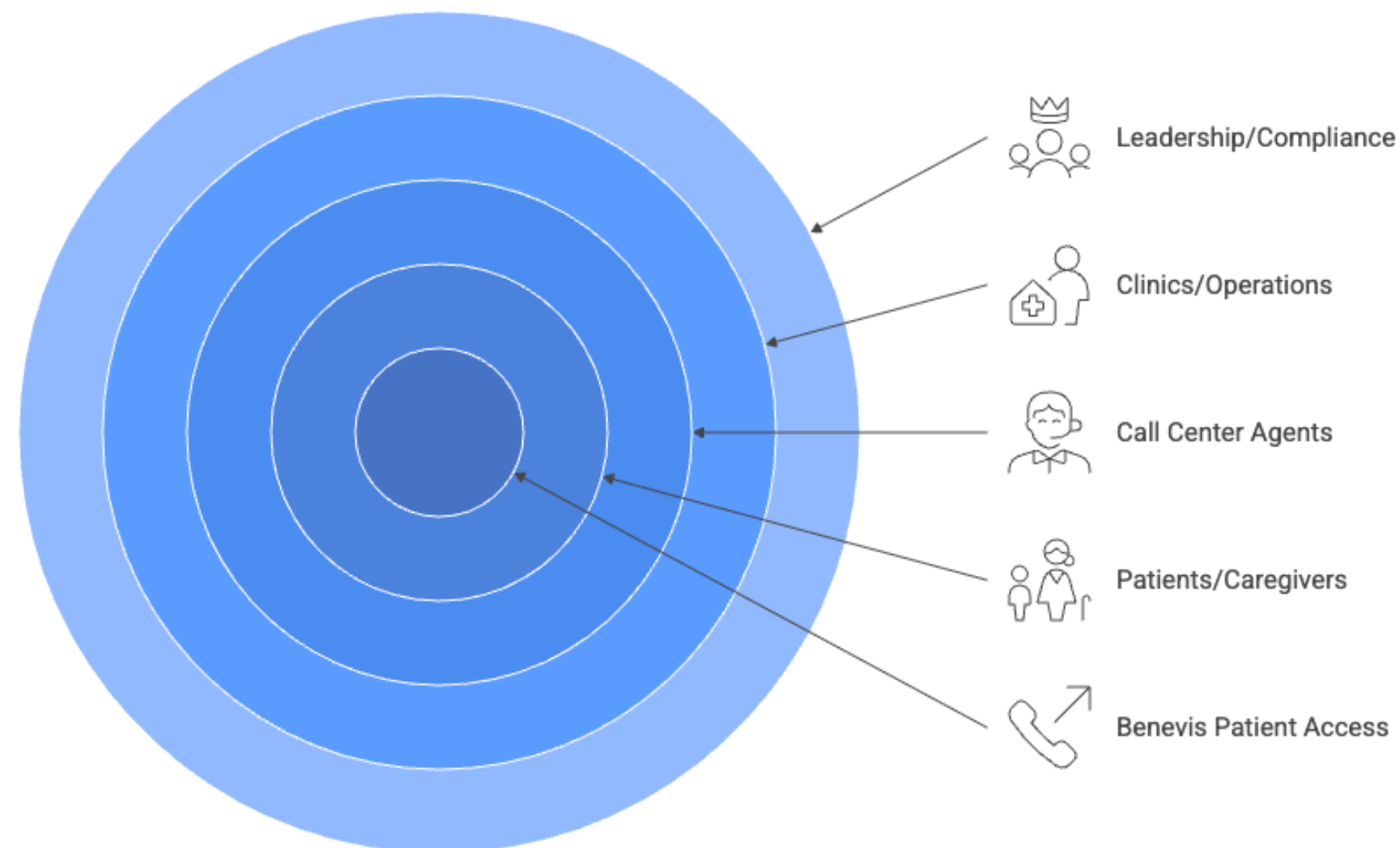
Patient experience & accessibility (waits/abandonment reduce access)



Stakeholder Impact

- **Patients & caregivers:** delays reduce access to care
- **Call-centre agents:** high routine volume → fatigue + inconsistency
- **Clinic staff/operations:** scheduling disruptions across locations
- **Leadership/compliance:** accountable for trust + privacy + rollout risk

Benevis Patient Access Stakeholder Map



Constraints and Reasons

Key Constraints (External & Risk Factors)



Technology & Data:

Fragmented systems and inconsistent data across locations reduce AI accuracy and make it difficult to scale automation reliably



Compliance & Privacy:

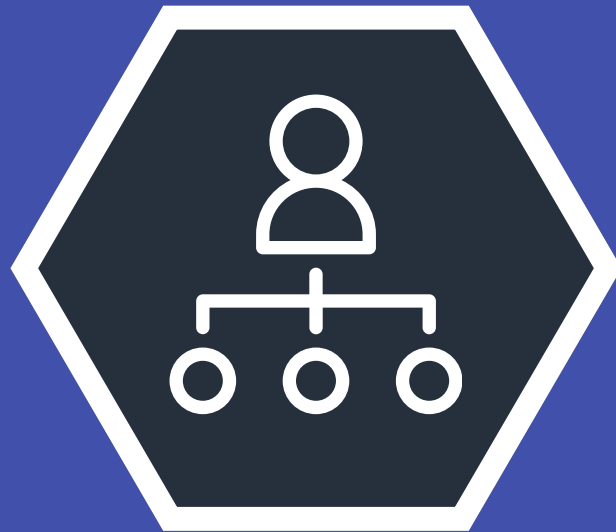
Strict healthcare regulations (HIPAA) require secure data handling and continuous monitoring, increasing implementation complexity and risk



Trust & Ethics:

Patients depend on human interaction and empathy; over-reliance on AI may reduce trust, especially in sensitive or complex situations

Internal Constraints & Limitations



Organizational Constraints

Limited training, resistance to change, and cross-functional coordination challenges slow down AI adoption and implementation



Financial & Operational Constraints

Tight margins and unpredictable call volumes make it difficult to invest in large-scale solutions or expand staffing

Evaluation of Strategic Alternatives



Full AI Automation

- Fully replaces human agents with AI
- High efficiency and scalability
- High risk (errors, poor patient experience)



Hybrid AI + Human

- AI handles routine tasks while humans handle complex cases
- Balanced efficiency and patient care
- Lower risk and strong compliance



Data-First Optimization

- Improve data and systems before AI
- Supports long-term accuracy
- Slow impact, does not address current call volume

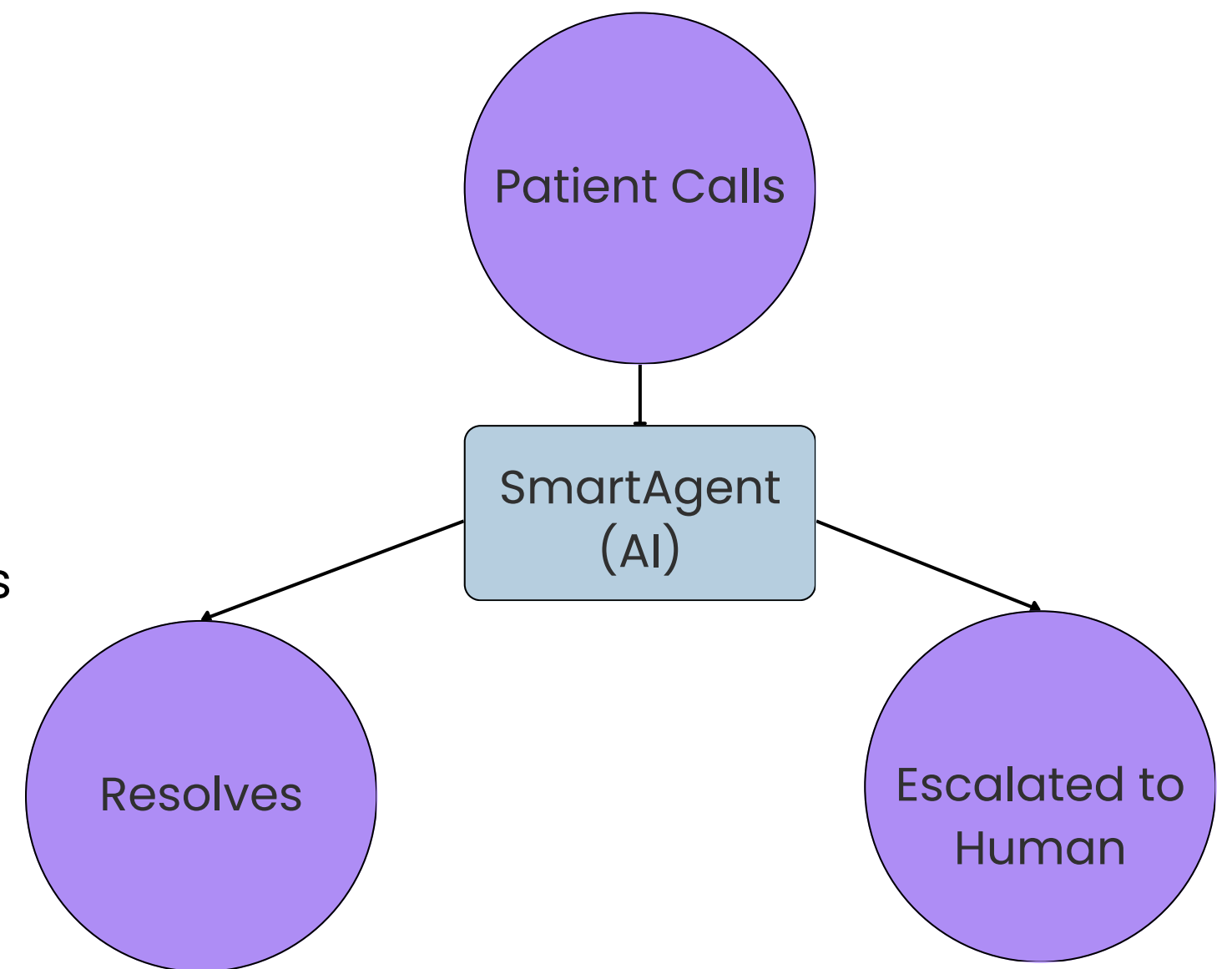
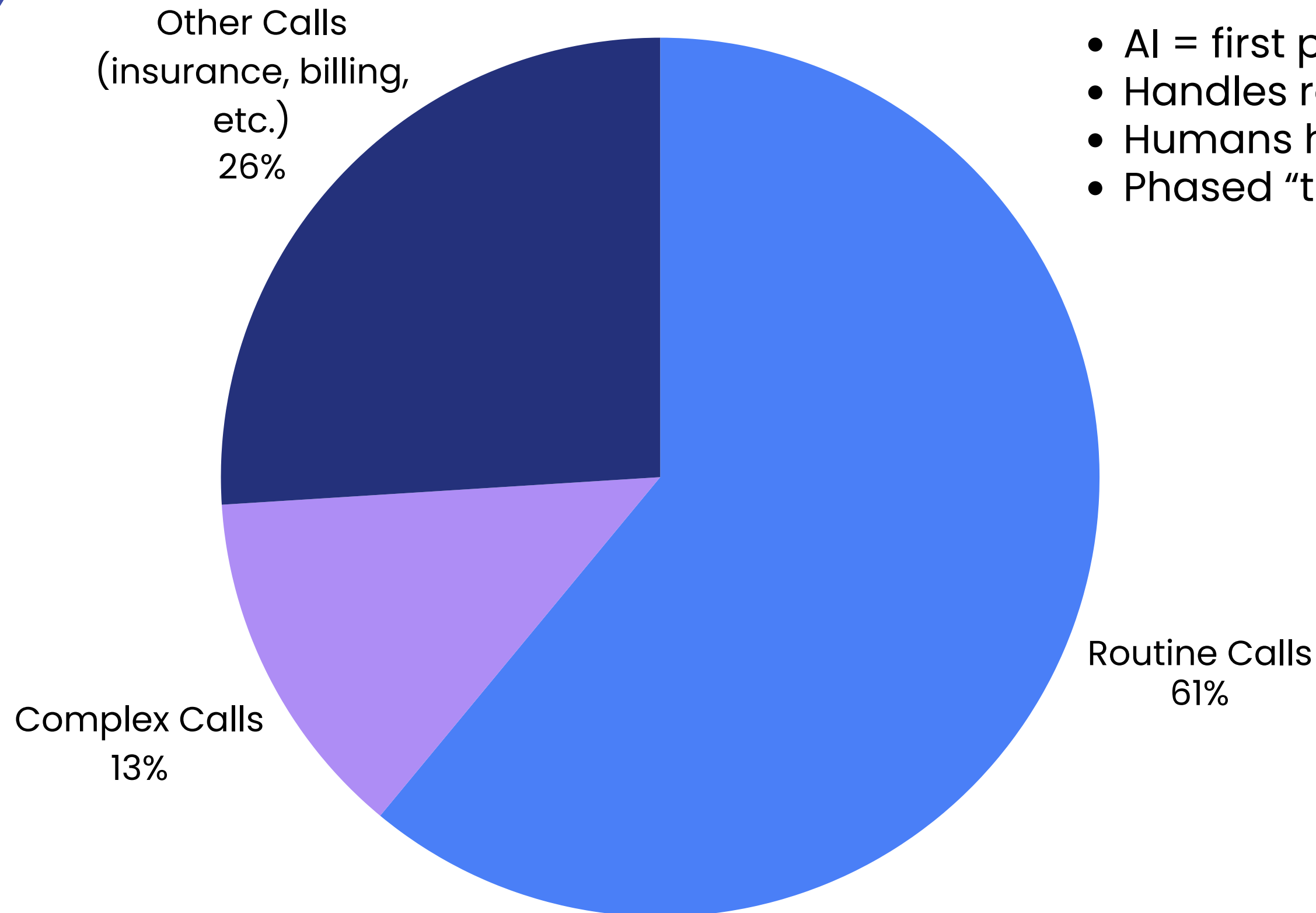


Outsourcing Operations

- External providers handle calls
- Quick short-term relief
- Low control, high data privacy risk

Best Solution: Hybrid AI + Human Model

- AI = first point of contact (“front door”)
- Handles routine calls (appointments, FAQs)
- Humans handle complex & sensitive cases
- Phased “test & learn” rollout



How It Works & Impact



Process

1. AI answers call
2. Identifies intent
3. Resolves OR escalates
4. Learns from interactions



Impact



\$1M+ annual savings



Handles peak call volumes



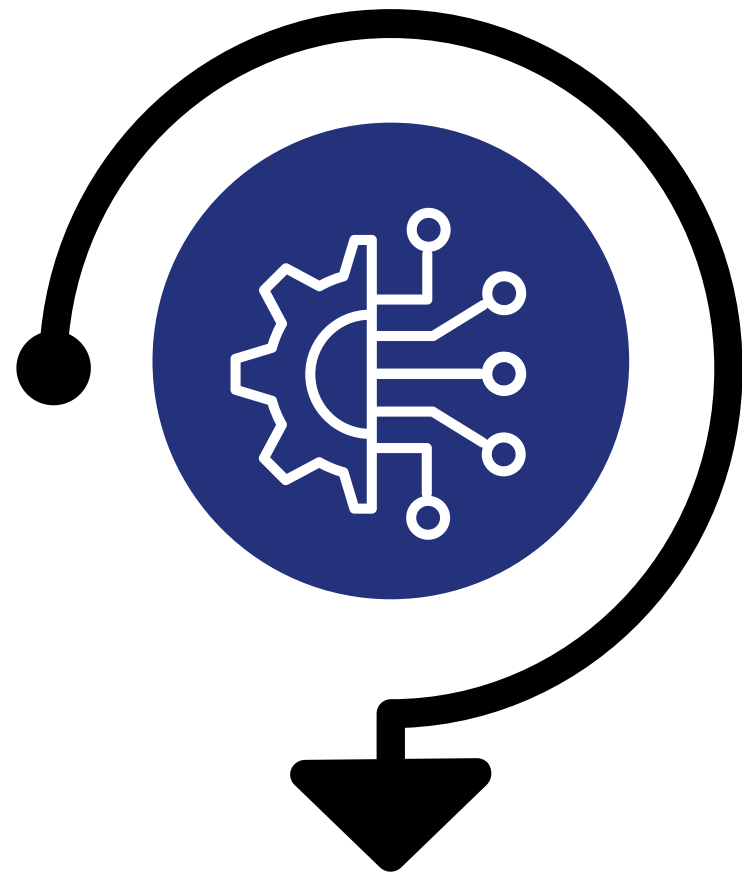
24/7 access



Faster service, lower wait times

Human-in-the-loop ensures safety, trust, and compliance.

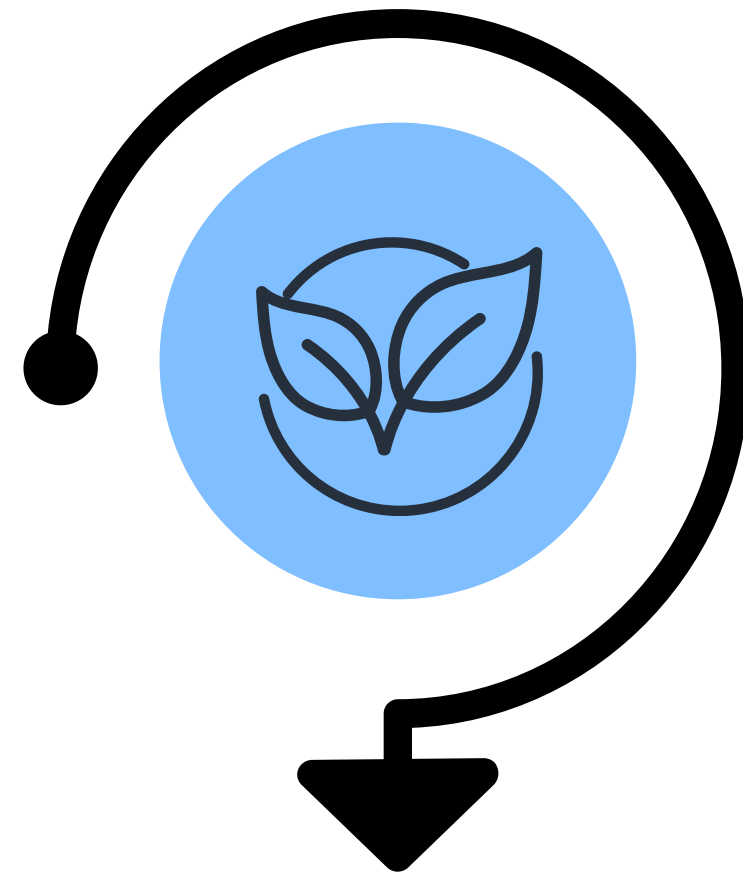
Phased Implementation Roadmap



Phase 1: Foundation

0-3 Months

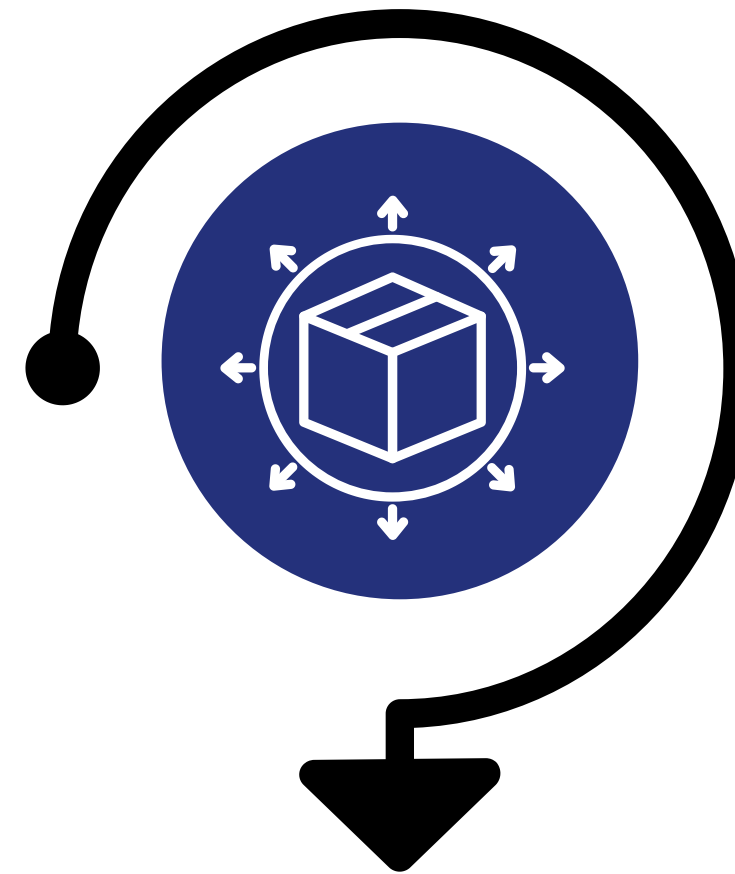
Data + integration
setup



Phase 2: Pilot

3-6 months

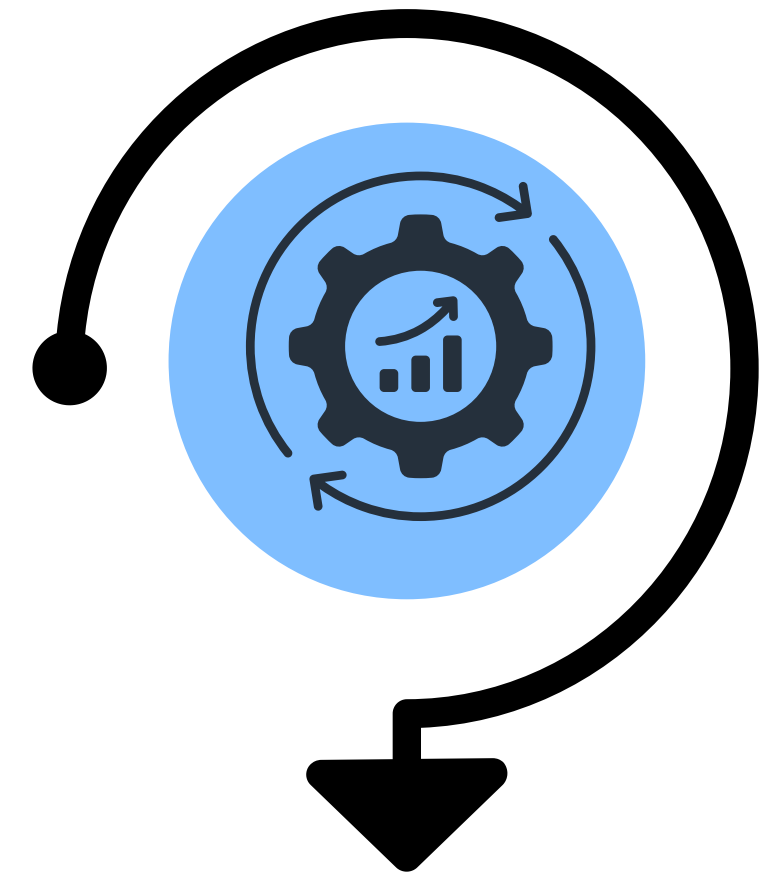
Test AI in selected
locations



Phase 3: Scale

6-12 months

Expand to 60–75%
of calls

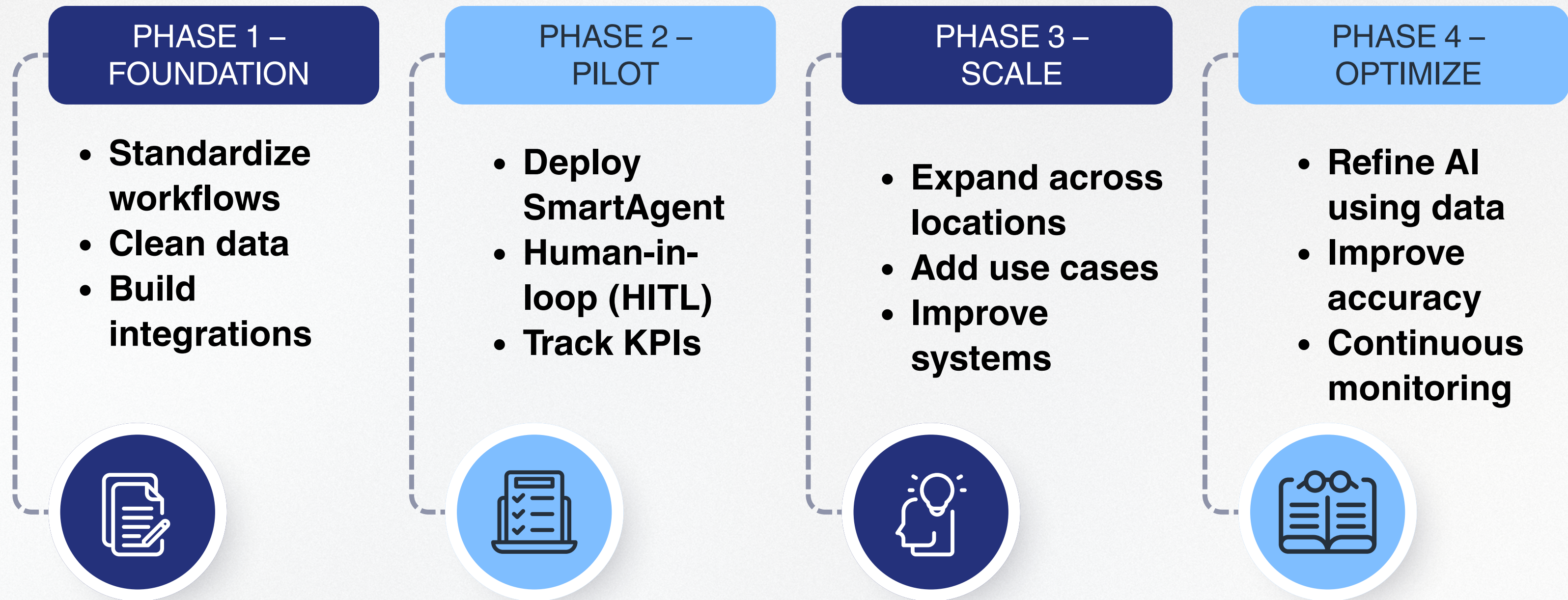


Phase 4: Optimize

12+ months

Improve
performance over
time

HOW THE SOLUTION IS IMPLEMENTED



Scale only after performance meets defined thresholds

Resource Allocation & Financial Impact

Resource Allocation



Technology

SmartAgent platform
Data integration & APIs
Performance dashboards

Human

Staff training for AI workflows
AI oversight roles
Cross-functional teams
(IT, Ops, Compliance)

Financial

One-time: integration, data setup,
training
Ongoing: platform + monitoring costs

Financial Impact & Business Value



- 75% calls automated
- ~1.2M calls annually
- \$1.85M annual ROI
- \$1.2M+ net savings
- ~6 months break-even

Implementation Roadmap



Immediate (0–2 months)

- Standardize data and workflows
- Establish governance and compliance



Short-Term (3–6 months)

- Pilot AI for appointment scheduling
- Monitor KPIs (automation, wait time, satisfaction)

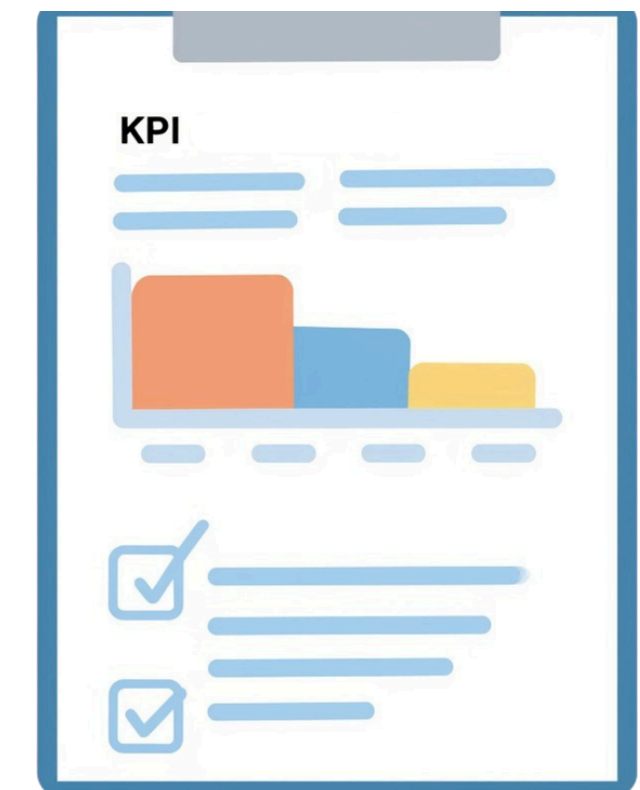
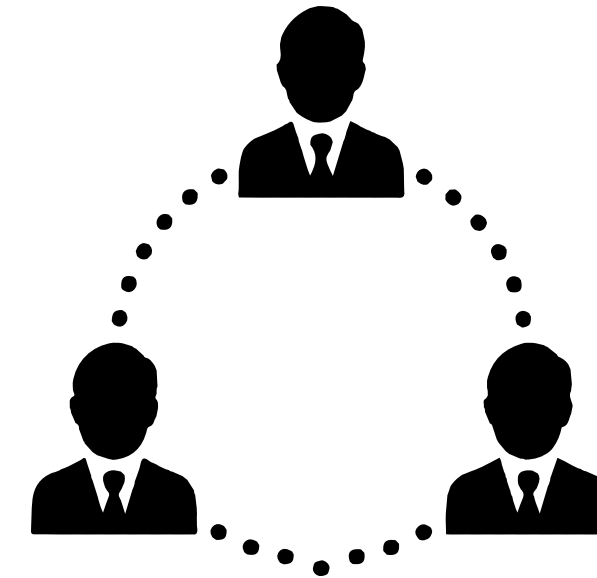


Medium-Term (6–12 months)

- Scale AI across locations
- Expand use cases (billing, reminders)

Governance & Key Success Factors

- Clear ownership (IT, Operations, Compliance)
- Human-in-the-loop for risk control
- Continuous monitoring (KPIs, dashboards)
- Staff training and change management
- Strong vendor coordination



Conclusion

- Current model is not scalable
- Hybrid model balances efficiency and risk
- Improves patient access and experience
- Enables sustainable, long-term growth



Thank You
For Your Attention